

SPEAK BEFORE THE REVEAL – GENERAL PRACTICE CHEAT SHEET

Consultation reference guide · speak these before you stop the recording

[Warning this cheat sheet was created by a Psychiatrist and requires GP review. Use it as an example and consider generating your own]

Speak your reasoning aloud while the scribe is still recording, so that your examination, your read of this patient and your plan, not the AI’s reconstruction, anchor the record. Finish the consultation, dictate the prompts below, and only then stop the recording. Work through them and skip what does not apply. The scribe reliably captures what was said in the room: the history, the numbers you read out, the advice you gave. It cannot capture what you saw, what you already knew about this patient, or what you decided in silence. Those are the items below.

The terms listed are descriptive vocabulary only, labels for observations you have already made and decisions you have already reached. They are not thresholds, decision rules or diagnostic criteria. This sheet tells you what to call it, never what you saw, or what it means.

OBSERVATION

Examination and observation “On examination I found...”	
General impression	How they look set against how they usually look · distress, pallor, breathlessness at rest · gait and mobility coming into the room · self-care and grooming · whether they strike you as well or unwell · affect and engagement · who came with them.
Focused examination	The systems you actually examined and what you found · observations you took yourself and their context · the site, character and extent of any abnormality · what you inspected, palpated or auscultated.
Point of care findings	What you saw down the otoscope or the dermatoscope, on the ECG, the spirometer, the dipstick · your reading of it, not only the value · images taken and what they showed.
Children and infants	Alertness and interaction · feeding and hydration · work of breathing · rash and its distribution · the parent’s level of concern and how it compares with previous visits.
What you chose not to examine	An examination deliberately deferred and why · a chaperone offered or declined · an examination the patient declined.
Negatives you sought	The specific findings you actively looked for and did not find. An unmentioned negative reads as a finding never sought.

Preserves: Direct physical observation, which no scribe can hear. **Guards against:** An examination reconstructed from conversational reassurance, or silently absent from the note.

INTERPRETATION

This patient against their own baseline “Knowing this patient, what is different today is...”	
Continuity is the thing you hold that the scribe does not. It has no field in the transcript.	
Consider How today compares with the last time you saw them · their usual level of function and where it sits now · the pattern across recent attendances rather than this one alone · what you know from the family, the file or the last decade that was never said aloud today · what has changed at home, at work or in their supports · whether the concern is theirs, a relative’s, or yours.	
Preserves: Longitudinal knowledge of a person, held by you and recorded nowhere. Guards against: A consultation written as a first presentation, because the transcript holds no history you did not repeat out loud.	

Problem representation and why now “My working problem representation is...”	
Consider A one-line summary carrying its qualifiers · acute or chronic · sudden or gradual · progressive or resolving · single system or multisystem · why they came today rather than last week or next month · the concern they arrived with, as distinct from the one you addressed · what else was raised and deliberately deferred · how the problems interact in multimorbidity · the social and occupational context bearing on the medical problem · whether the label is yours or inherited from a letter, a previous entry, or the patient.	
Preserves: Integrative synthesis, and the reason this presentation is happening now. Guards against: A problem list assembled from surface content, or a diagnosis carried forward unexamined.	

SYNTHESIS

Differential and what I excluded “The alternatives I am holding open are... and what I have excluded is...”	
Consider The serious diagnoses you actively considered in a low-prevalence setting · the feature that discriminates each competing hypothesis · the red flags you asked about and did not find · what you have excluded and by what means, whether by examination, by test, or by the course so far · what you are using time itself to exclude · drug-related and withdrawal causes · what remains hypothesis and what is established.	
Preserves: Lateral diagnostic thinking, and the negatives you actively sought. Guards against: Anchoring on the likely diagnosis; a pertinent negative lost because absence of mention reads as absence of finding.	

Key uncertainty “I am least sure of...”	
Consider Competing explanations still open · missing information such as old notes, hospital letters, prior imaging, an accurate medication history or collateral · what would change the plan · what you will reassess at the next visit and against what · your actual confidence in the working diagnosis · whether the consultation ran out of time before it ran out of problem.	
Preserves: Calibrated doubt and a live differential. Guards against: Declarative notes whose confidence conceals the gaps.	

Plan and its reasoning *“The plan, and the reasoning behind it, is...”*

Consider Each investigation with the question it answers and what each possible result would change · medication rationale, including the endpoint you intend and what you will monitor · what was discussed with the patient and what they understood · an option they declined and their reasoning · referral with the specific question asked, not only the destination · what you have asked them to do before the next visit · the interval to review, and why that interval.

Preserves: *Actions tied directly to the reasoning that produced them.*

Guards against: *A task list assembled from spoken instructions with the reasoning stripped out.*

Deliberate non-action *“What I am deliberately not doing, and why, is...”*

Consider Not prescribing, not imaging, not referring, not testing · watchful waiting and the point at which you would act · an antibiotic deliberately withheld, and what you explained · a result you are choosing not to chase · a medication deferred or deprescribed · an investigation whose burden or yield does not justify it · treatment the patient has declined.

Preserves: *The reasoning behind clinical restraint.*

Guards against: *Silence recorded as oversight, since nothing happens and so nothing is transcribed.*

Safety netting and follow-up *“What I have asked them to watch for, and when to come back, is...”*

Consider The specific changes you asked them to watch for · the timeframe you gave · who to contact, in hours and after hours · what you said would happen if it worsens · what you asked them to do if the treatment does not work · the appointment made, or deliberately not made, and why · who else has been informed · the review you would want even if they feel better.

Preserves: *The verbal contingency plan, usually the last thing said and the first thing lost.*

Guards against: *Advice given at the door, unrecorded, and unavailable to whoever sees them next.*

Worked example

“On examination I found *her well but tired, no pallor, chest clear, abdomen soft with mild suprapubic tenderness and no loin pain; I looked for and did not find a fever or vomiting. Knowing this patient, what is different today is that she has come in twice in six weeks for the same thing, which she has not done in the eight years I have known her. My working problem representation is recurrent lower urinary tract symptoms in an otherwise well woman, and the reason she is here today is not the symptoms themselves but her worry about what keeps causing them. The alternatives I am holding open are a partially treated infection and a non-infective cause of the same symptoms; what I have excluded is upper tract involvement on today's examination, and I asked about the red flags for anything more serious and found none. The plan, and the reasoning behind it, is to send the specimen before treating this time rather than after, because the recurrence is the thing I want explained. What I am deliberately not doing is starting another empirical antibiotic today, and I have explained to her why that is a decision rather than an omission. What I have asked her to watch for, and when to come back, is fever, loin pain or vomiting, in which case today and not next week; I have booked her for review once the result is back, however she feels. I am least sure whether this is genuinely recurrent infection or has been one unresolved episode all along.”*

Blank prompt card *Glance and dictate*

Examination: *On examination I found... General impression · focused systems · point of care · negatives sought*

Their baseline: *Knowing this patient, what is different today is... last visit · usual function · pattern · context*

Problem representation: *My working problem representation is... qualifiers · why now · what was deferred*

Differential & exclusions: *Held open... actively excluded... and by what · red flags sought · what time will exclude*

Key uncertainty: *I am least sure of...*

Plan & reasoning: *Each action, the question it answers, and what would change it · referral question · review interval and why*

Deliberate non-action: *What I am not doing, and why · what was explained · review point*

Safety netting: *What to watch for · by when · who to call · what if it worsens*

Adapted from Table 1, “Speak Before the Reveal”, for general practice. Verbalising these steps before the recording ends keeps the reasoning in active generation rather than passive editing, and reviewing is not the same skill as authoring.

Generated using the Speak Before the Reveal reference guide skill. Method: Jurblum M. Speak Before the Reveal: Seven Solutions to Governing AI Scribe Deskilling Risk in Psychiatry and Mental Health Services. Preprint, 28 July 2026. <https://doi.org/10.2139/ssrn.7317778> The underlying narration practice originates with Dr Michelle Adams.

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